

RIVERSIDE FAMILY MEDICAL CLINIC

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OUTPATIENT CLINICAL VISIT NOTE

Patient Name:	Jordan A. Whitfield	Date of Visit:	09/01/2026
Date of Birth:	03/14/1989 (Age 37)	MRN:	SYN-004471
Sex:	Male	Attending Physician:	Dr. Lena Ortiz, MD
Encounter Type:	Office Visit - New Complaint	Department:	Family Medicine / ENT Referral

CHIEF COMPLAINT

Facial pain/pressure, nasal congestion, and purulent nasal discharge for 10 days.

HISTORY OF PRESENT ILLNESS

Patient is a 37-year-old male presenting with a 10-day history of progressively worsening bilateral maxillary facial pain and pressure, most pronounced over the cheeks and around the eyes, which worsens on bending forward. He reports thick, yellow-green nasal discharge, nasal congestion, and post-nasal drip. Associated symptoms include reduced sense of smell (hyposmia), headache localized to the frontal region, mild dental pain in the upper teeth, ear fullness, sore throat from post-nasal drainage, and a persistent daytime cough that is worse at night when lying down. He notes low-grade fever (Tmax 100.9°F / 38.3°C) two days ago. Symptoms initially began as a common cold approximately 12 days ago and were improving, but worsened again after day 5 ("double worsening" pattern), raising concern for secondary bacterial sinusitis. No epistaxis, no visual changes, no periorbital swelling, no neck stiffness, and no confusion. Patient denies recent air travel or swimming. No prior sinus surgery. Symptoms have not improved with over-the-counter decongestants and saline rinses.

PAST MEDICAL HISTORY, MEDICATIONS & ALLERGIES

Past Medical History:	Seasonal allergic rhinitis, mild intermittent asthma
Past Surgical History:	None
Current Medications:	Fluticasone nasal spray (as needed), Albuterol inhaler (as needed)
Allergies:	No known drug allergies (NKDA)
Social History:	Non-smoker, occasional alcohol use, works as an accountant
Family History:	Non-contributory; father with seasonal allergies

REVIEW OF SYSTEMS

ENT: Positive for nasal congestion, purulent rhinorrhea, facial pressure, hyposmia, post-nasal drip, sore throat, and ear fullness. Denies epistaxis or hearing loss.

Respiratory: Positive for cough, worse when recumbent. Denies shortness of breath or wheezing.

Constitutional: Positive for low-grade fever and fatigue. Denies chills or night sweats.

Neurological: Positive for frontal headache. Denies confusion, vision changes, or neck stiffness.

Dental: Positive for mild maxillary tooth discomfort involving the upper molars.

Ophthalmologic: Denies eye pain, redness, swelling, or visual disturbance.

VITAL SIGNS

Temperature	99.8°F (37.7°C)	Heart Rate	84 bpm
Blood Pressure	122/78 mmHg	Respiratory Rate	16/min
SpO2	98% on room air	Weight / BMI	82 kg / 24.6

PHYSICAL EXAMINATION

General: Alert, mildly uncomfortable, no acute distress.

HEENT: Bilateral tenderness to palpation over the maxillary sinuses and frontal sinuses. Anterior rhinoscopy reveals erythematous, edematous nasal mucosa with thick purulent yellow-green discharge in the middle meatus bilaterally. Nasal turbinates are swollen. Positive "double sickening" history reported by patient. Transillumination of maxillary sinuses reduced bilaterally. Post-nasal purulent drainage visualized on posterior oropharynx exam. Mild erythema of the posterior pharynx. Tympanic membranes intact bilaterally with mild retraction, no effusion. No periorbital edema or erythema; extraocular movements intact; no proptosis (negative for orbital complication).

Neck: Supple, no lymphadenopathy, no meningismus.

Cardiovascular: Regular rate and rhythm, no murmurs.

Respiratory: Clear to auscultation bilaterally, no wheezes, rales, or rhonchi.

Neurological: Alert and oriented x3, no focal deficits, cranial nerves II-XII grossly intact.

DIAGNOSTIC FINDINGS

Nasal Endoscopy: Purulent secretions noted draining from the middle meatus, consistent with maxillary and anterior ethmoid sinus involvement. Mucosal edema present. No nasal polyps identified.

CT Sinuses (limited, non-contrast) - Impression: Mucosal thickening and air-fluid levels within the bilateral maxillary sinuses and anterior ethmoid air cells, consistent with acute bacterial rhinosinusitis. Frontal and sphenoid sinuses are clear. No evidence of orbital or intracranial extension.

Laboratory: WBC 11.8 x10⁹/L (mildly elevated), CRP 22 mg/L (elevated). Rapid strep antigen: negative.

ASSESSMENT

Primary Diagnosis: Acute Bacterial Sinusitis (Rhinosinusitis) — ICD-10: J01.90

Affected Sinuses:	Bilateral maxillary and anterior ethmoid sinuses
Severity:	Moderate
Differential Diagnoses Considered:	Viral upper respiratory infection (less likely given >10-day duration and double-worsening pattern), allergic rhinitis exacerbation, dental abscess (less likely, no localized dental swelling), migraine (less likely given exam findings).

Clinical presentation is consistent with acute bacterial sinusitis, supported by symptom duration greater than 10 days without improvement, the "double sickening" pattern, bilateral maxillary facial pain and pressure worsened by bending forward, purulent nasal discharge visualized on exam, maxillary sinus tenderness, hyposmia, and imaging demonstrating mucosal thickening with air-fluid levels in the maxillary and anterior ethmoid sinuses. Mildly elevated WBC and CRP support an underlying bacterial inflammatory process. No signs of orbital or intracranial complication at this time.

PLAN

- Start Amoxicillin-Clavulanate 875 mg/125 mg orally twice daily for 7 days.
 - Intranasal corticosteroid: Fluticasone propionate nasal spray, 2 sprays each nostril once daily, continued.
 - Saline nasal irrigation (e.g., neti pot or squeeze bottle) twice daily to promote sinus drainage.
 - Analgesic/antipyretic: Ibuprofen 400 mg every 6-8 hours as needed for pain and fever.
 - Encourage adequate hydration and rest.
 - Return to clinic or seek urgent care if symptoms worsen, if fever exceeds 102°F, or if periorbital swelling, vision changes, severe headache, or neck stiffness develop (signs of orbital or intracranial spread).
 - Follow-up appointment in 7-10 days to assess treatment response; refer to Otolaryngology (ENT) if symptoms fail to improve or recur frequently (consider chronic sinusitis workup).
 - Patient education provided regarding expected course of acute bacterial sinusitis and antibiotic adherence.
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Electronically signed by: **Dr. Lena Ortiz, MD** — Family Medicine
Date/Time: 09/01/2026, 10:42 AM

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